UUID:8982CE18-FA5A-4882-AAC4-F2AC6ED60C49
TCGA-49-AAR4-01A-PR

Redacted

[redacted]

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INTERPRETATION AND DIAGNOSIS:

1,2. LEFT UPPER LOBE AND LEFT UPPER LOBE #2:

SPECIMEN TYPE:
Wedge resection and completion lobectomy

ICD-O-3
Adenocarcinoma NOS 8140/3
Site: (L) Lung, upper lobe C34.1
JW 5/20/14

TUMOR SITE:
Left upper lobe

HISTOLOGIC TYPE:
Adenocarcinoma, Nos

TUMOR SIZE:
Greatest dimension: 3.0 cm

LOCATION, SIZE AND HISTOLOGIC TYPE OF
ADDITIONAL SEPARATE TUMOR NODULES, IF ANY:
None identified

HISTOLOGIC GRADE:
G2: Moderately differentiated

LYMPH NODES:
Metastatic carcinoma in 2 of 17 lymph nodes (see parts 4&6)

DIRECT EXTENSION OF TUMOR:
Visceral pleura (supported by Movats stain)

EXTENT OF INVASION
PRIMARY TUMOR:
pT2: Tumor invades visceral pleura

REGIONAL LYMPH NODES:
pN2: Metastasis in ipsilateral mediastinal or subcarinal lymph nodes

DISTANT METASTASIS:
pMX: Cannot be assessed

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MARGINS:
Margins uninvolved by invasive carcinoma

VENOUS/ARTERIAL (LARGE VESSEL) INVASION:
Indeterminate

LYMPHATIC (SMALL VESSEL) INVASION:
Indeterminate

ADDITIONAL FINDINGS
Respiratory bronchiolitis

3. LYMPH NODE, STATION 9 (EXCISION): ONE (1) LYMPH NODE, NEGATIVE FOR TUMOR.

4. LYMPH NODE, LEVEL 10 (EXCISION): METASTATIC ADENOCARCINOMA INVOLVING ONE (1) OF ONE (1) LYMPH NODE.

5. LYMPH NODE, LEVEL 12 (EXCISION): ONE (1) LYMPH NODE, NEGATIVE FOR TUMOR.

6. LYMPH NODE, L4 (DISSECTION): METASTATIC ADENOCARCINOMA INVOLVING ONE (1) OF FIVE (5) LYMPH NODES.

7. LYMPH NODE, LEVEL 7 (EXCISION): ONE (1) LYMPH NODE, NEGATIVE FOR TUMOR.

8. LYMPH NODES, L9 (DISSECTION): TWO (2) LYMPH NODES AND ASSOCIATED FIBROADIPOSE TISSUE, NEGATIVE FOR TUMOR.

9. LYMPH NODE, LEVEL 5 (EXCISION): ONE (1) LYMPH NODE, NEGATIVE FOR TUMOR.

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GROSS DESCRIPTION

PART #1: FS: LEFT UPPER LOBE
Resident Pathologist:

FROZEN SECTION DIAGNOSIS:
Staff Pathologist:

Other Pathologists / PAs:

Left upper lobe: Non-small cell carcinoma.

The specimen is received fresh labeled [redacted], and designated left upper lobe. It consists of a 63.7 gms, 13.5 x 5 x 4.2 cm wedge shaped lung tissue. The margin is closed with staples. The staples are removed and the margins are inked black. The pleural surface has a 2.5 cm area of stellate puckering with a firm area of lung parenchyma suspicious for tumor below it. The pleura is inked black. Upon serial sectioning, there is a 3 x 1.7 x 1.5 cm white tan poorly circumscribed stellate subpleural mass, that is 0.3 cm from the stapled margin. The mass does not appear to involved the pleural surface. A representative section of the mass is submitted for frozen section. The remaining lung is red brown and non-crepitant. Representative sections are submitted with approximately 80% of the tumor submitted.

SUMMARY OF SECTIONS:

1	- FSC	- 1	
M	- A-D	- 1	(PUCKERED PLEURA WITH SUBPLEURAL MASS)
1	- E	- 1	(PERPENDICULAR TUMOR CLOSEST TO RESECTION MARGIN)
2	- F-G	-= 1 EA.	(NON-INVOLVED LUNG)
8	-TOTAL-	8	

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PART #2: FS: LEFT UPPER LOBE #2
Resident Pathologist:

FROZEN SECTION DIAGNOSIS:
Staff Pathologist:
Other Pathologists / PAs:

Left upper lobe #2: Bronchial margin and one (1) lymph node, negative for tumor.

The specimen is received fresh labeled [redacted] and deignated left upper lobe #2. It consists of a 320 gms, 15 x 12 x 4 cm completion lobectomy specimen. The lung is diffusely non-crepitant. The pleura is slightly hemorrhagic and mottled with anthracotic pigmentation. No mass is palpated within this portion of specimen. Additionally, no pleura puckering is identified. There is a stapled margin consistent with the area from where the initial wedge resection was taken. There is a 1.2 cm portion of bifurcating bronchus present which appears patent. No tumor is identified within the lumen. A shave of this bronchi are taken. Multiple red brown hilar lymph nodes are are identified ranging in size from 0.5 to 1.4 cm. None of these appear grossly positive. The entire pleural margin is inked black. Upon serial sectioning, the lung has a uniform congested red brown with focal emphysematous changes and diffuse anthracotic pigmentation. The hilar vessels are remarkable. No masses or lesions are identified. Representative sections are submitted.

SUMMARY OF SECTIONS:

1	- FSC	- 1	
1	- A	- 3	
2	-B-C	- 1 EA.	(PERPENDICULAR OF GROSSLY NORMAL LUNG) INCLUDING MARGIN OF WEDGE RESECTION)
M	-D-F	- 1 EA.	(ADDITIONAL LEFT UPPER LOBE LUNG)
1	- G	- 2	(ONE POTENTIAL HILAR LYMPH NODE BISECTED)
1	- H	- 4	(ONE POTENTIAL HILAR LYMPH NODE)
1	- I	- 2	(ONE POTENTIAL HILAR LYMPH NODES, ? POS. BISECTED)
10	- TOTAL	- 17	

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PART #3: STATION 9
Resident Pathologist:

The specimen is received fresh labeled [redacted] and designated station 9. It consists of one unoriented fragment of yellow, to red brown fibrofatty tissue measuring 0.4 x 0.4 x 0.2cm. Upon dissection two potential lymph nodes are identified with a red brown color. One measures 4 x 0.2 x 0.2 cm. and the other lymph nodes measuring 0.2 x 0.2 x 0.2 cm. Neither lymph nodes appear grossly positive. The specimen is submitted in its entirety.

SUMMARY OF SECTIONS:
1 -A - 2
1 -TOTAL - 2

PART #4: LEVEL 10
Resident Pathologist:

The specimen is received fresh labeled [redacted] and designated level 10. It consists of one unoriented nodular fragment of yellow red tan fibrofatty tissue measuring 2.5 x 1.2 x 0.8 cm. Upon lymph node dissection, one tan white to tan black lymph node is identified. The lymph node appears to have a diffuse anthracotic pigment within it and multiple firm nodular areas consistent with metastatic tumor.
has harvested a portion of the tumor for tumor bank. The lymph node measures 1.9 x 1.6 x 1 cm. The specimen is serially sectioned and submitted in its entirety.

SUMMARY OF SECTIONS:
1 - A - 2
1 - B - 2 (ONE POSITIVE LEVEL 10 NODE SERIALLY SECTIONED)
2 -TOTAL - 4

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PART #5: LEVEL 12
Resident Pathologist:

The specimen is received fresh labeled [redacted], and designated level 12. It consists of one unoriented and severely cauterized fragment of yellow red to black fibrofatty tissue measuring 0.5 x 0.4 x 0.4 cm. Grossly one potential lymph node measuring 0.4 cm in greatest dimension is identified. The specimen is submitted in its entirety.

SUMMARY OF SECTIONS:
1 - A - 1 (ONE POTENTIAL LEVEL 12 LYMPH NODE)
1 -TOTAL- 1

PART #6: L4
Resident Pathologist:

Th specimen is received fresh labeled [redacted] and designated L4. It consists of multiple fragments of yellow tan to red brown, black fibroadipose tissue measuring in aggregate 1.5 x 1 x 0.5 cm. Grossly, upon dissection, six potential lymph nodes are identified measuring in size from 0.2 x 0.4 cm in greatest dimension. Multiple lymph nodes have a black pigmentation consistent with anthracosis. However, none of the nodes appears grossly positive. The specimen is submitted in its entirety in lens paper.

SUMMARY OF SECTIONS:
1 - A - 6 (SIX POTENTIAL L4 NODES)
1 -TOTAL - 6

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PART #7: LEVEL 7
Resident Pathologist:

The specimen is received fresh labeled [redacted], and designated level 7. It consists of one unoriented soft yellow pink tan to dark red black fibroadipose tissue measuring 1.3 x 0.8 x 0.4 cm. Upon lymph node dissection, one potential lymph node is identified consistent it has a dark red black parenchyma consistent with anthracosis. The node does not appear grossly positive. The specimen is submitted in its entirety.

SUMMARY OF SECTIONS:

PART #8: L9
Resident Pathologist:

The specimen is received fresh labeled [redacted] and designated L9. It consists of a one unoriented fragment of yellow tan red to dark red to black fibroadipose tissue measuring 1.5 x 1 x 0.4 cm. Upon dissection, one potential lymph node is identified.

SUMMARY OF SECTIONS:
1 - A - 1 (ONE NODE BISECTED)
1 - B - 3 (SMALLER POTENTIAL NODE AND SOFT TISSUE)
2 -TOTAL-5

PART #9: LEVEL 5
Resident Pathologist:

The specimen is received fresh labeled [redacted] and designated level 5. It consists of one unoriented fragment of dark red brown to black tissue measuring 1.0 x 0.5 x 0.5 cm. The potential lymph node does not appear grossly positive. The specimen is bisected to reveal a soft fleshy tan to red brown variegated parenchyma. The entire specimen is submitted.

SUMMARY OF SECTIONS:
1 - A - 2

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1 - TOTAL - 2

[redacted]

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(End of Report) printed

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		✓
Case is (circle): QUALIFIED / DISQUALIFIED		
Reviewer Initials MIE Date Reviewed: 12/12/13		

lw 1/18/14